This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the supply of Mysimba (Naltrexone 8mg / Bupropion 90mg) prolonged-release tablets for the treatment of obesity

Patient Group Direction, version 007, issued 30 September 2026. Supersedes Version 006, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE / NICE CKS Guidelines for obesity

Overview

Obesity is a chronic disease affecting a significant proportion of the population. Clinical assessment should include measurement of weight, height and calculation of body mass index (BMI), waist circumference, and assessment of cardiovascular risk.

- **Management approach:**The primary management of obesity involves lifestyle intervention with a reduced-calorie diet and increased physical activity. Pharmacological interventions, including Mysimba, should be considered as adjunctive therapy in patients who have not achieved adequate weight loss through lifestyle measures alone.

Pharmacological treatment

Mysimba (naltrexone/bupropion combination) is a pharmacological option, as an adjunct to lifestyle measures, for weight management in adults with obesity who have not achieved adequate weight loss with lifestyle intervention alone.

- **Indication:**Adjunct to a reduced-calorie diet and increased physical activity for weight management in adults with initial BMI ≥30 kg/m² or ≥27 kg/m² in the presence of at least one weight-related comorbidity.
- **Duration of treatment:**Treatment should be reviewed at 16 weeks. If weight loss of at least 5% has not been achieved by 16 weeks, Mysimba should be discontinued.

Patient Group Direction

for the supply/administration of

Mysimba (Naltrexone 8mg / Bupropion 90mg) prolonged-release tablets

for the treatment of

obesity

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover

	this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Adjunct to a reduced-calorie diet and increased physical activity for weight management in adults with initial BMI ≥30 kg/m² or ≥27 kg/m² with at least one weight-related comorbidity (such as type 2 diabetes, hypertension, dyslipidaemia, or sleep apnoea).
Inclusion criteria	Age 18 to 74 years (inclusive) BMI ≥30 kg/m² or ≥27 kg/m² with at least one weight-related comorbidity Documented failed weight loss attempt through lifestyle intervention (reduced-calorie diet and physical activity) for at least 3 months Able to provide informed written consent Not currently pregnant or breast-feeding; females of childbearing potential should use effective contraception Resting blood pressure <140/90 mmHg (if ≥140/90 mmHg, refer to GP for management before initiating Mysimba)
Exclusion criteria	Uncontrolled hypertension (blood pressure ≥140/90 mmHg) or history of significant cardiovascular disease Aged 75 years and over (the SmPC does not recommend Mysimba in patients over 75 years; refer to the GP) Seizure disorders or any history of seizures Head trauma with loss of consciousness within the last 12 months CNS tumour or history of CNS tumour Abrupt discontinuation (withdrawal) of alcohol or benzodiazepines; current alcohol withdrawal; or concurrent use of benzodiazepines Current opioid use or opioid dependence; do not use within 7-10 days of opioid discontinuation Use of monoamine oxidase inhibitors (MAOIs) or within 14 days of MAOI discontinuation History of eating disorders (bulimia nervosa or anorexia nervosa) Severe hepatic impairment (Child-Pugh Class C) Moderate or severe renal impairment, or end-stage renal failure (eGFR below 60 mL/min/1.73m²): refer to the GP. The reduced dose

	that the SmPC gives for eGFR 15 to 59 mL/min/1.73m² is a prescriber decision and is not supplied under this PGD Any history of bipolar disorder Current depression, suicidal ideation, or any history of suicide attempt Currently taking any other product containing bupropion (for example Zyban) or naltrexone Known hypersensitivity to naltrexone, bupropion, or any excipients Angle-closure glaucoma
Cautions	Hypertension: Monitor blood pressure regularly (at least monthly initially). Blood pressure must be below 140/90 mmHg at every supply; if 140/90 mmHg or above, do not supply and refer to the GP. Bupropion may elevate blood pressure; discontinue if sustained elevation occurs. Hepatic impairment: Use with caution in mild to moderate hepatic impairment. Avoid in severe impairment. Renal impairment: eGFR below 60 mL/min/1.73m² excludes (see exclusion criteria). Mild renal impairment (eGFR 60 to 89 mL/min/1.73m²) needs no dose adjustment; use with caution. Ask about kidney disease and check any recent eGFR result. Where the patient has a condition that affects the kidneys (for example diabetes or hypertension) or reports kidney disease and no eGFR result is available, obtain one from the GP before supply. Elderly patients aged 65 to 74 years: use with caution, as the SmPC advises; they may be more sensitive to adverse effects and are more likely to have reduced renal function (see the renal caution). Patients aged 75 years and over are excluded (see exclusion criteria). Suicidal ideation and behaviour: Monitor for mood changes, depression, anxiety, and suicidal thoughts, particularly in the first weeks of treatment and following any dose adjustment. Seizure threshold: Bupropion lowers seizure threshold; avoid concurrent use of medications that lower seizure threshold. Caution in patients with CNS conditions, and where there is a history of head trauma with loss of consciousness more than 12 months ago (within the last 12 months excludes; see exclusion criteria). Blood glucose: May affect glucose control in diabetic patients; monitor blood glucose closely and adjust antidiabetic medication if necessary. Concurrent medications: Potential for drug interactions with medications metabolised by CYP2D6, antidepressants, and other CNS-active drugs; review all medications before initiating. Angle-closure glaucoma risk: Bupropion may increase intraocular pressure; caution in patients at risk of angle-closure glaucoma.

	Tachycardia: Monitor heart rate; bupropion may increase heart rate or cause tachycardia.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Mysimba tablets containing naltrexone hydrochloride 8mg and bupropion hydrochloride 90mg (prolonged-release)
Legal category	POM (Prescription Only Medicine)
Storage	Store below 30°C. Keep in original container.
Route/method of administration	Oral. Swallow tablets whole. Do not crush, chew, or divide tablets. Can be taken with or without food (avoid high-fat meals).
Dose and frequency	Titration over 4 weeks: • Week 1: One tablet once daily in the morning • Week 2: One tablet in the morning and one tablet in the evening • Week 3: Two tablets in the morning and one tablet in the evening • Week 4 onwards (maintenance): Two tablets in the morning and two tablets in the evening (maximum 4 tablets per day, 2 twice daily) After titration, the dose should be maintained at 4 tablets daily (2 tablets twice daily) unless dosage adjustment is required.
Quantity to be administered and/or supplied	Up to 120 tablets per supply (28-day supply at the full maintenance dose of 4 tablets daily). The titration month needs 70 tablets (7, 14, 21 and 28 in weeks 1 to 4). Supply no more than covers the period to the next review, and nothing beyond the 16-week maximum.
Maximum or minimum treatment period	Review at 16 weeks of treatment: the pharmacist weighs the patient and records the result. Discontinue if less than 5% of initial body weight has been lost. Maximum treatment period under this PGD: 16 weeks (the titration month and three maintenance months). Continuation beyond 16 weeks is by prescription from the GP or a specialist prescriber, not under this PGD. Follow-up: weight, blood pressure and pulse at 4 weeks (end of titration) and at 16 weeks; ask about mood, suicidal thoughts, seizures and symptoms of raised blood pressure at every supply, and stop and refer if any is present.
Adverse effects	Very common (≥1/10): Nausea, constipation, headache, insomnia Common (≥1/100 to <1/10): Vomiting, dry mouth, dizziness, anxiety,

	tremor, upper abdominal pain, tachycardia, hypertension, decreased appetite, rash, pruritus Uncommon (≥1/1,000 to <1/100): Seizures, fever, agitation, abnormal dreams, palpitations, sweating, dyspepsia Serious adverse effects (report immediately): Seizures, severe hypertension, hepatotoxicity (jaundice, elevated liver enzymes), severe allergic reactions (including angioedema), suicidal ideation or behaviour, severe mood changes

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • weight, blood pressure and pulse recorded at this visit, and the 16-week weight result • any eGFR result relied on, with its date • date of supply, dose, form, route and quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken • that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with Mysimba. Provide written information regarding the dose titration schedule, lifestyle modifications (reduced-calorie diet and physical activity), and emergency contact details.
Follow-up advice to be given to patient or carer	Swallow tablets whole; do not crush, chew, or divide them. Follow the dose titration schedule carefully over the first 4 weeks; do not increase the dose faster than prescribed. Take the medication as prescribed even if you do not see immediate

	results; weight loss may take several weeks to become apparent. Maintain a reduced-calorie diet and engage in regular physical activity as directed by your healthcare provider or dietitian. Report any mood changes, depression, anxiety, suicidal thoughts, or unusual behaviour immediately to your healthcare provider or emergency services. Monitor your blood pressure regularly at home if possible; report any persistent elevation to your GP. Do not consume excessive alcohol as it may increase the risk of seizures and adverse effects. Avoid high-fat meals as they may affect absorption; take with water. Do not stop the medication abruptly; discuss discontinuation with your healthcare provider. Inform all healthcare providers (including dentists) that you are taking Mysimba, as it may interact with other medications. If you experience persistent insomnia, headache, nausea, or constipation, discuss management options with your healthcare provider. Attend all follow-up appointments as scheduled, particularly the review at 16 weeks to assess weight loss and tolerability. Supply under this PGD stops at 16 weeks; any continuation is by prescription from your GP or a specialist prescriber. Report any allergic reactions (rash, itching, swelling) or chest pain to your healthcare provider or seek emergency care immediately.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance: Weight management in obesity
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises /			

clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
30/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Version and change record

Version: v007

Issued: 30 September 2026

Supersedes: Version 006, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v006

Issued: 14 September 2026

Supersedes: Version 005, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v005

Issued: 11 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Decision 51: age 75 years and over is now an exclusion (the SmPC does not recommend Mysimba over 75); the inclusion criterion reads age 18 to 74 years and the elderly caution now covers 65 to 74 years.

Decision 52: renal impairment with eGFR below 60 mL/min/1.73m² now excludes and is referred to the GP, because the reduced dose the SmPC gives for eGFR 15 to 59 is a prescriber decision; the renal caution now covers mild impairment (eGFR 60 to 89) and asks for an eGFR result where kidney disease is present or suspected, and the records row asks for any eGFR result relied on.

Decision 53: head trauma with loss of consciousness now excludes only where it occurred within the last 12 months; the seizure exclusion reads seizure disorders or any history of seizures, and older head trauma with loss of consciousness is a caution under the seizure threshold row.

Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Cover title corrected from administration to supply; the tablets are supplied.

Guidance summary: second-line wording removed so the summary does not read as requiring prior drug treatment, which the inclusion criteria do not require.

Exclusion criteria: the alcohol and benzodiazepine exclusion is rewritten as three clear items with the same scope.

Cautions: the hypertension caution now states the 140/90 mmHg limit that applies at every supply, matching the inclusion and exclusion criteria.

Quantity row: the titration month quantity (70 tablets) is stated and the open-ended adjustment sentence replaced with a supply-to-next-review rule within the 16-week maximum.

Follow-up advice: the patient is told that supply under this PGD stops at 16 weeks and continuation is by prescription.

Records row: the two run-together bullets were separated and punctuated (date of supply, dose, form, route and quantity supplied; adverse reactions and actions taken; supplied via PGD).

Records row: the under-18 record-retention bullet was removed because this PGD does not treat anyone under 18.

Records row: weight, blood pressure, pulse and the 16-week weight result added, as the treatment-period row already requires them to be recorded.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

A maximum treatment period is stated: 16 weeks under this PGD, continuation by prescription only. The previous version left the maximum to be 'determined based on continued clinical benefit', which is not a maximum. The 16-week weight review is assigned to the pharmacist and a follow-up schedule added.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

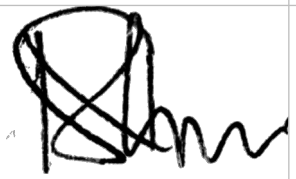
Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded.

Exclusion criteria: 'Bipolar disorder or major depressive disorder with suicidal ideation' read as excluding bipolar disorder only where suicidal ideation was present. The SmPC (4.3) contraindicates any history of bipolar disorder. Split into two exclusions.

Exclusion added: any other product containing bupropion (for example Zyban) or naltrexone, which the SmPC contraindicates and the previous version did not mention.

Signed on behalf of Get Real Health

Version v007, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.